Approval Date:

Aspirin Enteric-Coated Tablets Instructions

Please read the instructions carefully and use under the guidance of a physician.

Drug Name

Generic Name: Aspirin Enteric-coated Tablets

English name: Aspirin Enteric-Coated Tablets

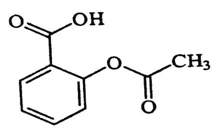
Pinyin: Asipilin Changrong Pian

Ingredients

The active ingredient of this product is 2-(acetoxy)benzoic acid.

Chemical name: 2-(Acetoxyl)benzoic acid

chemical structural formula：



Molecular formula: $C_9H_8O_4$

Molecular weight: 180.16

【shape and properties】

This product is an enteric-coated tablet, appearing white after removal of the coating.

【indication】

This product is a nonsteroidal anti-inflammatory drug (NSAID). Clinically, it can be used to prevent thrombosis, including transient ischemic attack (TIA), myocardial infarction (MI), atrial fibrillation, artificial heart valve complications, arteriovenous fistulas, or postoperative thrombosis. It is also indicated for the treatment of unstable angina pectoris. For antipyretic and analgesic purposes or in the management of rheumatic diseases, high-dose formulations of aspirin should be selected.

【specifications】

40mg

【usage and dosage】

For oral administration. The usual adult dosage is 40 mg to 300 mg (1 tablet to 7 tablets) per dose, once daily. For prophylaxis: generally 40 mg to 160 mg (1 tablet to 4 tablets) per day; for treatment: generally 300 mg (7 tablets) per day.

【untoward effect】

The higher the plasma drug concentration, the more pronounced the adverse reactions.

1、 Common gastrointestinal reactions include nausea, vomiting, epigastric discomfort, or pain, which typically resolve after discontinuation of the medication. Prolonged or high-dose administration may lead to gastrointestinal bleeding or ulceration.

2、 Allergic reactions: Occurred in 0.2% of patients, manifesting as asthma, urticaria, angioedema, or shock. These reactions predominantly occur in susceptible individuals, with rapid onset of dyspnea after medication administration; severe cases may be fatal, a condition termed aspirin-induced asthma. Some patients may develop the triple syndrome of aspirin allergy, asthma, and nasal polyps, which is often associated with genetic and environmental factors.

【taboo】

1、 Contraindicated in patients with known hypersensitivity to this product.

2、 The following situations should be disabled:

① Active ulcer disease or gastrointestinal bleeding caused by other reasons;

② Hemophilia or thrombocytopenia;

③ Patients with a history of allergy to aspirin or other nonsteroidal anti-inflammatory drugs (NSAIDs), particularly those presenting with asthma, neurovascular edema, or shock.

【 matters need attention 】

1、 Cross-allergic reaction. Allergy to this product may also lead to hypersensitivity to another salicylate-based or non-salicylate nonsteroidal anti-inflammatory drug (NSAID). However, this is not absolute, and the possibility of cross-allergy must be carefully considered.

2、 Interference with diagnosis:

1) May interfere with urine ketone test;

2) The fluorescence method for determining urinary 5-hydroxyindoleacetic acid (5-HIAA) may be interfered with by this substance.

3) The determination of vanillin-methyl-ammonium acid (VMA) may yield varying results depending on the methodology employed.

4) Since this drug inhibits platelet aggregation, it may prolong the bleeding time. Even a low dose of 40 mg/day can affect platelet function; however, no clinical reports have documented bleeding caused by such low doses (<150 mg/day).

5) Since this drug acts on the renal tubules, increasing potassium excretion, it may lead to hypokalemia.

6) Since this product competes with phenolsulfonophthalein for renal tubular excretion, it reduces the excretion of phenolsulfonophthalein (i.e., PSP excretion test).

3、 The following conditions require cautious use:

1) In cases of asthma and other allergic reactions;

2) Patients with glucose-6-phosphate dehydrogenase deficiency (this product may occasionally cause hemolytic anemia);

3) Gout (This product may interfere with the efficacy of other uricosuric agents and may cause uric acid retention at low doses);

4) Impaired liver function may exacerbate hepatic toxicity reactions and increase the tendency for bleeding. Patients with hepatic insufficiency or cirrhosis are more susceptible to adverse renal reactions.

5) Renal insufficiency carries an increased risk of renal toxicity.

6) Patients with thrombocytopenia.

[Use in Pregnant and Lactating Women]

This product readily crosses the placenta. Animal studies have demonstrated teratogenic effects when administered during the first three months of pregnancy; however, no such adverse reactions have been observed at standard therapeutic doses. The drug is excreted in breast milk.

[Pediatric Medication]

It is not yet clear.

[Geriatric Medication]

In elderly patients with decreased renal function, this product should be administered under the guidance of a physician.

【 drug interaction 】

1、 When used concomitantly with any drug that may induce hypoprothrombinemia, thrombocytopenia, impaired platelet aggregation function, or gastrointestinal ulcer bleeding, there is an increased risk of exacerbating coagulation disorders and inducing hemorrhage.

2、 Concomitant use with anticoagulants (such as dicoumarol and heparin) or thrombolytics (such as streptokinase and urokinase) may increase the risk of bleeding.

3、 Adrenocortical hormone drugs can increase the excretion of salicylates. When used concomitantly, the dosage of this drug should be increased if necessary to maintain its plasma concentration.

4、 It can enhance and accelerate the hypoglycemic effect of insulin or oral hypoglycemic agents.

5、 When co-administered with methotrexate (MTX), it reduces the binding of MTX to proteins and decreases its renal excretion, leading to elevated plasma drug concentrations and increased toxic effects.

6、 The uricosuric effect of probenecid or sulfinpyrazone may be diminished when these agents are administered concomitantly; this reduction becomes evident at a salicylate plasma concentration of 50 μg/mL and is more pronounced at concentrations exceeding 100–150 μg/mL. Additionally, probenecid decreases the renal clearance of salicylates, thereby increasing their plasma concentrations.

【 overdose 】

Manifestations of overdose poisoning:

① Mild manifestations, known as salicylism, include headache, dizziness, hearing loss, nausea, vomiting, diarrhea, somnolence, mental confusion, hyperhidrosis, deep and rapid breathing, polydipsia, involuntary movements of the hands and feet (more common in elderly individuals), and visual disturbances.

class 。

② Severe cases may present with hematuria, convulsions, hallucinations, severe psychiatric disturbances, dyspnea, and non-specific fever.

Pharmacology and Toxicology

1. Antithrombotic and antiplatelet aggregation effects: By inhibiting platelet cyclooxygenase, aspirin reduces the production of thromboxane A2 (TXA2) catalyzed by cyclooxygenase. TXA2 accelerates platelet aggregation in vivo. Low-dose aspirin primarily inhibits TXA2, thereby exhibiting potent antiplatelet aggregation and antithrombotic effects. At high doses, aspirin also suppresses the production of prostaglandin I2 (PGI2), which promotes platelet aggregation and thrombus formation.
2. Anti-inflammatory and anti-rheumatic effects: Acting on inflamed tissues, it exerts its anti-inflammatory effects by inhibiting the synthesis of prostaglandins or other substances that induce inflammatory responses (such as histamine), stabilizing lysosomal membranes, and suppressing the release of lysosomal enzymes.
3. Antipyretic effect: It reduces the body temperature of febrile patients with minimal impact on normal body temperature. This effect is achieved by inhibiting the synthesis and release of prostaglandins in the thermoregulatory center, thereby enhancing heat dissipation processes (such as dilation of cutaneous blood vessels and increased perspiration).
4. Analgesic effect: Primarily achieved by inhibiting the synthesis of prostaglandins and other substances that enhance pain sensitivity to mechanical or chemical stimuli (e.g., bradykinin, histamine), it belongs to the class of peripheral analgesics.

【 pharmacokinetics 】

This drug is largely absorbed in the upper small intestine, although enteric-coated tablets exhibit slower absorption. The protein binding rate is low; after hydrolysis, the salicylate's protein binding rate ranges from 65% to 90%. The binding rate decreases correspondingly at higher plasma concentrations. It remains low in patients with renal impairment and during pregnancy. The T1/2 is 15–20 minutes; at typical clinical doses achieving plasma concentrations of 5.4–9 mg/L, platelet aggregation is inhibited—e.g., a daily dose of 180 mg inhibits TXA2 synthase activity by 99%. The T1/2 of salicylates depends on the dose and urinary pH; a single low-dose administration results in an elimination half-life of approximately 2–3 hours.

This drug is rapidly hydrolyzed into salicylates in the gastrointestinal tract, liver, and bloodstream, followed by further metabolism in the liver. The primary metabolites are salicyluric acid and its glucuronide conjugates, with a small portion being oxidized to gentisic acid. Peak plasma concentrations are achieved 1–2 hours after a single administration. The drug is excreted via the kidneys as both conjugated metabolites and free salicylic acid.

【 store up 】

Seal and store in a dry place.

【 pack 】

Packaged in polyethylene plastic bottles, 100 pieces per bottle.

【 term of validity 】

Preliminary duration: 24 months.

【 operative norm 】

National Pharmaceutical Standard: WS-10001-(HD-0614)-2002

【 license number 】

National Drug Approval Number: H11021614

【 manufacturing enterprise 】

Company Name: Beijing Shuguang Pharmaceutical Co., Ltd.

Production Address: Guanzhuang, Jianguo Road, Chaoyang District, Beijing

Postal code: 100024

Phone number: 010-65459308

Fax number: 010-65459318

Website:www. bjsgyy. com